Panitumumab 6mg/kg and FOLFIRI Therapy-14 day

INDICATIONS FOR USE:

INDICATION	ICD10	Regimen Code	HSE approved reimbursement status*
First line treatment of adult patients with wild-type RAS metastatic colorectal cancer (mCRC).	C18	00448a	N/A
Second line treatment of adult patients with wild-type RAS mCRC who have received first-line fluoropyrimidine–based chemotherapy (excluding irinotecan).	C18	00448b	N/A

*This applies to post 2012 indications

TREATMENT:

The starting dose of the drugs detailed below may be adjusted downward by the prescribing clinician, using their independent medical judgement, to consider each patients individual clinical circumstances.

Treatment is administered every 14 days until disease progression or unacceptable toxicity develops.

Facilities to treat anaphylaxis MUST be present when systemic anti-cancer therapy (SACT) is administered.

Admin. Order	Day	Drug	Dose	Route	Diluent & Rate	Cycle
1	1	Panitumumab	6mg/kg	IV infusion	[1]100mL 0.9% NaCl over 60 minutes[2]using a 0.22 micron in-line filter	Every 14 days
[1]In 150ml over 90 minutes if dose > 1000mg. Final concentration should not exceed 10mg/mL. [2]If the first infusion is tolerated, then subsequent infusions may be administered over 30 to 60 minutes.						
Panitumumab is incompatible with glucose solutions. Ensure IV administration sets are flushed with 0.9% NaCl pre and post administration.						
2	1	Irinotecan	180mg/m^2	IV infusion	250mL 0.9% NaCl over 90 minutes	Every 14 days
3	1	Folinic Acid (Calcium leucovorin)	[3]400mg/m^2	IV infusion	250mL 0.9% NaCl over 2 hours	Every 14 days
4	1	5-Fluorouracil	400mg/m^2	IV BOLUS	Slow push through side arm of fast flowing drip	Repeat every 14 days
5	1	[4] 5-Fluorouracil	2400mg/m^2	Continuous IV infusion	Over 46 hours in 0.9% NaCl	Repeat every 14 days
[3]A dose of 200mg/m^2 of folinic acid may be considered.						
Irinotecan and leucovorin may be infused at the same time by using a y-connector placed immediately before the injection site. Irinotecan and leucovorin should not be combined in the same infusion bag.						
Folinic Acid *(Calcium Leucovorin)* must be administered prior to 5-Fluorouracil. It enhances the effects of 5-Fluorouracil by increasing 5-Fluorouracil binding to the target enzyme thymidylate synthetase.						

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[4]See dose modifications section for patients with identified partial dihydropyrimidine dehydrogenase (DPD) deficiency
Patients may suck on ice chips during the bolus injection of 5-Fluorouracil to reduce stomatitis.

Note: Administration volumes and fluids have been standardised to facilitate electronic prescribing system builds

ELIGIBILITY:

- Indications as above
- Wild type RAS tumours verified by a validated test method
- ECOG 0-2
- Adequate haematological, renal and liver status

CAUTION:

Use with caution in patients with

- Previous pelvic radiotherapy
- Recent MI
- Uncontrolled angina, hypertension, cardiac arrhythmias, congestive heart failure (CHF)
- In patients with baseline greater than 3 loose bowel movements (BM) per day (in patients without colostomy or ileostomy)

EXCLUSIONS:

- Hypersensitivity to panitumumab, irinotecan, folinic acid, 5-Fluorouracil or any of the excipients
- Patients with mutant RAS mCRC or unknown RAS mCRC status
- Baseline neutrophils $<2x10^9$/L and/or platelet count $<100x10^9$/L
- Renal impairment
- Hepatic impairment
- Patients with interstitial pneumonitis or pulmonary fibrosis
- Chronic bowel disease and/or bowel obstruction
- Pregnancy and breast feeding
- Known complete dihydropyrimidine dehydrogenase (DPD) deficiency

PRESCRIPTIVE AUTHORITY:

The treatment plan must be initiated by a Consultant Medical Oncologist.

TESTS:

Baseline tests:

- FBC, liver and renal profile
- ECG (if patient has compromised cardiac function)

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- DPD testing prior to first treatment with 5-Fluorouracil using phenotype and/or genotype testing unless patient has been previously tested
    - In patients with moderate or severe renal impairment, blood uracil levels used for dihydropyrimidine dehydrogenase (DPD) phenotyping should be interpreted with caution, as impaired kidney function can lead to increased uracil blood levels. Consequently, there is an increased risk for incorrect diagnosis of DPD deficiency, which may result in under dosing of 5-Fluorouracil or other fluoropyrimidines, leading to reduced treatment efficacy. Genotype testing for DPD deficiency should be considered for patients with renal impairment.

Regular tests:

- FBC, liver and renal profile prior to each cycle
- Post treatment: monthly electrolytes, magnesium, calcium for 2 months after last panitumumab treatment
- INR tests if patient is on warfarin as clinically indicated

Disease monitoring:

Disease monitoring should be in line with the patient's treatment plan and any other test/s as directed by the supervising Consultant.

DOSE MODIFICATIONS:

- Consider a reduced starting dose in patients with identified partial DPD deficiency
    - Initial dose reduction may impact the efficacy of treatment
    - In the absence of serious toxicity, subsequent doses may be increased with careful monitoring
- Any dose modification should be discussed with a Consultant
- Panitumumab or FOLFIRI therapy may be delayed independently of each other and dosing may continue with either component but consideration should be given to the timings of further treatment
- Irinotecan should be administered after appropriate recovery of all adverse events to grade 0 or 1 NCI-CTC grading and when treatment-related diarrhoea is fully resolved
- At the start of a subsequent infusion of therapy, the dose of irinotecan and 5-Fluorouracil, should be decreased according to the worst grade of adverse events observed in the prior infusion
- Treatment should be delayed by 1 to 2 weeks to allow recovery from treatment-related adverse events

The following dose reductions should be used when calculating FOLFIRI dose reductions for patients with toxicities

Table 1: Dose Reduction Levels for All Toxicities

	Dose Level 0	Dose Level -1	Dose Level -2	Dose Level -3
Irinotecan	180 mg/m^2	150 mg/m^2	120 mg/m^2	Discontinue
Folinic Acid *(Calcium Leucovorin)*	400 mg/m^2	400 mg/m^2	400 mg/m^2	Discontinue
5-Fluorouracil bolus	400 mg/m^2	320 mg/m^2	260 mg/m^2	Discontinue
5-Fluorouracil infusion	2400 mg/m^2	1900 mg/m^2	1500mg/m^2	Discontinue

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Note: Folinic acid is delayed or omitted if bolus 5-Fluorouracil is delayed or omitted

Table 2: Dose Modification of FOLFIRI for Haematological Toxicity

Prior to a Cycle (DAY 1)	Toxicity		Dose Level for Subsequent Cycles	
	Grade	**ANC ($x10^9$/L)**	**Irinotecan**	**5-Fluorouracil**
• If ANC< 1.5 on Day 1 of cycle, hold treatment, weekly FBC, maximum of 2 weeks. • ANC ≥ 1.5 within 2 weeks, proceed with treatment at the dose level noted across from the **lowest ANC** result of the delayed week(s). • If ANC remains <1.5 after 4 weeks discontinue treatment.	1	≥ 1.5	Maintain dose level	Maintain dose level
	2	1.0-1.49	Maintain dose level	Maintain dose level
	3	0.5-0.99	↓ 1 dose level	↓ 1 dose level
	4	<0.5	↓2 dose levels	↓2 dose levels
	Grade 4 neutropenia and grade≥2 fever		↓2 dose levels	↓2 dose levels
	Grade	**Platelets ($x10^9$/L)**	**Irinotecan**	**5-Fluorouracil**
• If platelets < 75 on Day 1 of cycle, hold treatment, weekly FBC, maximum of 2 weeks. • Platelets ≥ 75 within 2 weeks, proceed with treatment at the dose level noted across from the **lowest platelets** result of the delayed week(s). • If platelets remain <75 after 2 weeks, discontinue treatment.	1	≥ 75	Maintain dose level	Maintain dose level
	2	50-74.9	Maintain dose level	Maintain dose level
	3	10-49.9	↓ 1 dose level	↓ 1 dose level
	4	<10	↓ 2 dose levels	↓ 2 dose levels
The use of granulocyte colony-stimulating factor (G-CSF) may be considered.				

Renal and Hepatic Impairment:

Table 3: Dose modification in patients with renal or hepatic impairment

Drug	Renal impairment	Hepatic impairment			
Panitumumab	No studies have been performed in patients with renal impairment.	No studies have been performed in patients with hepatic impairment.			
Irinotecan	No dose reduction needed, however use with caution as no information in this setting.	Irinotecan is contraindicated in patients with bilirubin levels >3xULN.			
5-Fluorouracil	Consider dose reduction in severe renal impairment only.	**Bilirubin (micromol/L)**		**AST**	**Dose**
		<85		<180	100%
		>85	or	>180	CI
		Clinical decision. Moderate hepatic impairment; reduce initial dose by 1/3. Severe hepatic impairment, reduce initial dose by 1/2. Increase dose if no toxicity.			

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Management of adverse events:

Table 4: Dose modification of FOLFIRI for adverse events

Prior to a Cycle (DAY 1)	Grade of Toxicity	Dose Level for Subsequent Cycles	
		Irinotecan	5-Fluorouracil
Diarrhoea • ≥ Grade 2, hold treatment max of 2 weeks. • < Grade 2 within 2 weeks proceed with treatment at the dose level noted across from the **highest** grade experienced. • Remains ≥ Grade 2 after 2 weeks, discontinue treatment.	1 and 2	Maintain dose level	Maintain dose level
	3	↓1 dose level	↓ 1 dose level
	4	↓2 dose levels	↓2 dose levels
Stomatitis • ≥ Grade 2, hold treatment max of 2 weeks. • < Grade 2 within 2 weeks proceed with treatment at the dose level noted across from the **highest** grade experienced. • Remains ≥ Grade 2 after 2 weeks, discontinue treatment.	1 and 2	Maintain dose level	Maintain dose level
	3	Maintain dose level	↓1 dose level
	4	Maintain dose level	↓2 dose levels

Table 5: Dose modification of panitumumab for skin reactions.

Occurrence of skin symptom(s): ≥ grade 3	Administration of panitumumab	Outcome	Dose regulation
Initial occurrence	Hold 1 or 2 doses	Improved (< grade 3)	Continue infusion at 100% original dose
		Not recovered	Discontinue
2nd occurrence	Hold 1 or 2 doses	Improved (< grade 3)	Continue infusion at 80% of original dose
		Not recovered	Discontinue
3rd occurrence	Hold 1 or 2 doses	Improved (< grade 3)	Continue infusion at 60% of original dose
		Not recovered	Discontinue
4th occurrence	Discontinue		

Local skin care policy for the prevention and treatment of EGFR-inhibitor adverse skin reactions should be instigated as appropriate.

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Table 6: Dose modification of panitumumab for adverse events

Adverse reaction	Recommended dose modification
Infusion reaction	Decrease infusion rate of panitumumab and maintain lower rate for subsequent infusions
Severe infusion reaction	Discontinue
Interstitial lung disease	Discontinue

SUPPORTIVE CARE:

EMETOGENIC POTENTIAL:

- As outlined in NCCP Classification Document for Systemic Anti-Cancer Therapy (SACT) Induced Nausea and Vomiting – available on the NCCP website

Panitumumab: Low **(Refer to local policy).**
Irinotecan: Moderate **(Refer to local policy).**
5-Fluorouracil: Low **(Refer to local policy).**

For information:
Within NCIS regimens, antiemetics have been standardised by the Medical Oncologists and Haemato-oncologists. Information is available in the following documents:

- NCCP Supportive Care Antiemetic Medicines for **Inclusion in NCIS** (Medical Oncology) - Available on the NCCP website
- NCCP Supportive Care Antiemetic Medicines for **Inclusion in NCIS** (Haemato-oncology) - Available on the NCCP website

PREMEDICATIONS:

Prophylactic atropine sulphate 250micrograms subcutaneously – see adverse effects below.
Atropine should not be used in patients with glaucoma. (See Adverse Effects/Regimen specific complications below).

OTHER SUPPORTIVE CARE:

Anti-diarrhoeal treatment **(Refer to local policy).**
Patients should be made aware of the risk of delayed diarrhoea occurring more than 24 hours after the administration of irinotecan and at any time before the next cycle.

- As soon as the first liquid stool occurs, the patient should start drinking large volumes of beverages containing electrolytes and an appropriate anti-diarrhoeal therapy must be initiated immediately
- The currently recommended anti-diarrhoeal treatment consists of high doses of loperamide (4mg for the first intake and then 2 mg every 2 hours)
- This therapy should continue for 12 hours after the last liquid stool and should not be modified
- In no instance should loperamide be administered for more than 48 consecutive hours at these doses, because of the risk of paralytic ileus, nor for less than 12 hours

Patients should be warned about the potential for dizziness or visual disturbances which may occur within 24 hours following the administration of irinotecan, and advised not to drive or operate machinery if these symptoms occur.

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ADVERSE EFFECTS / REGIMEN SPECIFIC COMPLICATIONS:

The adverse effects listed are not exhaustive. Please refer to the relevant Summary of Product Characteristics for full details.

- **Neutropenia:** Fever or other evidence of infection must be assessed promptly and treated appropriately.

Panitumumab

- **Infusion-related reactions:**
    - In cases of mild or moderate infusion-related reaction, the infusion rate may be decreased and maintained at the lower rate in all subsequent infusions.
    - Occurrence of a severe infusion-related reaction requires immediate and permanent discontinuation of panitumumab therapy and may necessitate emergency treatment.
    - Hypersensitivity reactions occurring more than 24 hours after infusion have been reported. Patients should be warned of the possibility of such a late onset and instructed to contact their physician if symptoms occur.
- **Respiratory disorders:** Interstitial lung disease (ILD) has been observed with EGRF inhibitors. Treatment should be withheld in the event of onset or worsening respiratory symptoms. If ILD is confirmed, treatment should be discontinued.
- **Acute renal failure**: This has been observed in patients who develop severe diarrhoea and dehydration.
- **Skin reactions**: This is the main adverse reaction of panitumumab. Refer to local policy for skin care regime and to Table 5 under Dose Modifications for management of treatment if patient experiences skin reactions.
- **Electrolyte disturbances:** Hypomagnesaemia, hypokalaemia or hypocalcaemia may occur. Electrolyte repletion is recommended, as appropriate.
- **Ocular toxicities:** Patients presenting with signs and symptoms suggestive of keratitis such as acute or worsening: eye inflammation, lacrimation, light sensitivity, blurred vision, eye pain and/or red eye should be referred promptly to an ophthalmology specialist. If a diagnosis of ulcerative keratitis is confirmed, treatment should be interrupted or discontinued. If keratitis is diagnosed, the benefits and risks of continuing treatment should be carefully considered.

Irinotecan

- **Acute cholinergic syndrome:** If acute cholinergic syndrome appears (defined as early diarrhoea and various other symptoms such as sweating, abdominal cramping, lacrimation, myosis and salivation) atropine sulphate (250 micrograms subcutaneously) should be administered unless clinically contraindicated. Caution should be exercised in patients with asthma. In patients who experienced an acute and severe cholinergic syndrome, the use of prophylactic atropine sulphate is recommended with subsequent doses of irinotecan**.**
- **Diarrhoea:** Irinotecan induced diarrhoea can be life threatening and requires immediate management.
    - Diarrhoea (early onset) - see acute cholinergic syndrome above.
    - Diarrhoea (late onset):
        - Irinotecan induced diarrhoea can be life threatening and requires immediate management
        - In monotherapy, the median time of onset of the first liquid stool was on day 5 after the infusion of irinotecan
        - Patients with an increased risk of diarrhoea are those who had previous abdominal/pelvic

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radiotherapy, those with baseline hyperleucocytosis, those with performance status ≥2 and women
    - In patients who experience severe diarrhoea, a reduction in dose is recommended for subsequent cycles
    - The SmPC provides guidelines on when hospitalisation for the management of diarrhoea is recommended
- **Extravasation:** Irinotecan causes pain and tissue necrosis if extravasated**. (Refer to local extravasation guidelines).**
- **Gilbert's Syndrome:** Increases the risk of irinotecan-induced toxicity. A reduced initial dose should be considered for these patients.
- **Respiratory disorders:** Severe pulmonary toxicity has been reported rarely. Patients with risk factors should be monitored for respiratory symptoms before and during irinotecan therapy.

5-Fluorouracil

- **Myocardial ischaemia and angina:** Cardiotoxicity is a serious complication during treatment with 5-Fluorouracil. Patients, especially those with a prior history of cardiac disease or other risk factors, treated with 5-Fluorouracil, should be carefully monitored during therapy.
- **DPD deficiency:** DPD is an enzyme encoded by the DPYD gene which is responsible for the breakdown of fluoropyrimidines. Patients with DPD deficiency are therefore at increased risk of fluoropyrimidine-related toxicity, including for example stomatitis, diarrhoea, mucosal inflammation, neutropenia and neurotoxicity. Treatment with 5-Fluorouracil, capecitabine or tegafur-containing medicinal products is contraindicated in patients with known complete DPD deficiency. Consider a reduced starting dose in patients with identified partial DPD deficiency. Initial dose reduction may impact the efficacy of treatment. In the absence of serious toxicity, subsequent doses may be increased with careful monitoring. Therapeutic drug monitoring (TDM) of 5-Fluorouracil may improve clinical outcomes in patients receiving continuous 5-Fluorouracil infusions.
- **Hand-foot syndrome (HFS),** also known as palmar-plantar erythrodysaesthesia (PPE) has been reported as an unusual complication of high dose bolus or protracted continuous therapy for 5-Fluorouracil.

DRUG INTERACTIONS:

- No formal drug-drug interaction studies have been conducted with panitumumab. Panitumumab should not be administered in combination with IFL chemotherapy or with bevacizumab-containing chemotherapy.
- Risk of drug interactions causing decreased concentrations of irinotecan with CYP3A inducers.
- Risk of drug interactions causing increased concentrations of irinotecan with CYP3A inhibitors. Patients should also be counselled with regard to consumption of grapefruit juice.
- Prochlorperazine should be avoided on the same day as irinotecan treatment due to the increased incidence of akathisia.
- Marked elevations of prothrombin time and INR have been reported in patients stabilized on warfarin therapy following initiation of 5-Fluorouracil regimens.
- Concurrent administration of 5-Fluorouracil and phenytoin may result in increased serum levels of phenytoin.
- 5-Fluorouracil is contraindicated in combination with brivudin, sorivudin and analogues as these are potent inhibitors of the 5-Fluorouracil-metabolising enzyme DPD.

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- Caution should be taken when using 5-Fluorouracil in conjunction with medications which may affect DPD activity.
- Current drug interaction databases should be consulted for more information.

REFERENCES:

1. Peeters M, Price TJ et al. Randomized Phase III Study of Panitumumab with Fluorouracil, Leucovorin, and Irinotecan (FOLFIRI) Compared With FOLFIRI Alone As Second-Line Treatment in Patients With Metastatic Colorectal Cancer. J Clin Oncol 2010; 28:4706-4713
2. BCCA Protocol Summary for Palliative Combination Chemotherapy for Metastatic Colorectal Cancer Using Irinotecan, Fluorouracil and Leucovorin GIFOLFIRI
3. Dosage Adjustment for Cytotoxics in Renal Impairment January 2009; North London Cancer Network
4. Dosage Adjustment for Cytotoxics in Hepatic Impairment January 2009; North London Cancer Network.
5. NCCP Classification Document for Systemic Anti-Cancer Therapy (SACT) Induced Nausea and Vomiting. V3 2021. Available at: https://www.hse.ie/eng/services/list/5/cancer/profinfo/chemoprotocols/nccp-classification-document-for-systemic-anti-cancer-therapy-sact-induced-nausea-and-vomiting.pdf
6. HPRA Direct Healthcare Professional Communication. 5-Fluorouracil (i.v.), capecitabine and tegafur containing products: Pre-treatment testing to identify DPD-deficient patients at increased risk of severe toxicity. Accessed Aug 2020 Available at: https://www.hpra.ie/docs/default-source/default-document-library/important-safety-information-from-marketing-authorisation-holders-of-products-containing-5-fluorouracil-(i-v-)-capecitabine-and-tegafur-as-approved-by-the-hpra.pdf?sfvrsn=0
7. Panitumumab (VECTIBIX®) Summary of Product Characteristics. Last updated: 11/09/2018 Accessed Nov 2021. Available at: https://www.ema.europa.eu/en/documents/product-information/vectibix-epar-product-information_en.pdf
8. Irinotecan (CAMPTO®) Summary of Product Characteristics. HPRA last updated: 16/07/2021. Accessed Nov 2021. Available at: https://www.hpra.ie/img/uploaded/swedocuments/Licence_PA0822-212-001_16072021151724.pdf
9. Fluorouracil 50 mg/ml solution. Summary of Product Characteristics. HPRA last updated: 16/04/2021. Accessed: Nov 2021. Available at:https://www.hpra.ie/img/uploaded/swedocuments/Licence_PA2315-091-001_16042021165722.pdf

From 27/08/2026, this regimen will be reviewed on an 'As Required' basis. Please refer to the version control box for information, and the footer for date of last review. If you wish to suggest any updates to this regimen, please contact oncologydrugs@cancercontrol.ie

Version control

Version	Date	Amendment	Approved By
1	23/10/2017		Prof Maccon Keane
2	09/10/2019	Reviewed. Standardisation of treatment table. Update of exclusions, drug interactions, emetogenic potential.	Prof Maccon Keane

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		Removal of company resources.	
3	01/09/2020	Updated exclusion criteria, baseline testing, dose modifications and adverse events with respect to DPD deficiency as per DHPC from HPRA June 2020 Updated Adverse events regarding palmar-plantar erythrodysaesthesia	Prof Maccon Keane
4	21/12/2021	Reviewed. Added to exclusions.	Prof Maccon Keane
4a	21/11/2023	Formatting changes and grammatical corrections.	NCCP
4b	03/03/2025	Additional wording added to baseline testing section.	NCCP
5	27/08/2026	Addition of links in emetogenic potential section. Amendment to review schedule.	NCCP

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